

Behavior Interpretation & Family Support Toolkit

Internal Booklet Prototype

A caregiver resource for understanding difficult behavior
across FASD, neurodevelopmental differences, and family support needs.

Contents

1. Cover / Orientation
2. Behavior as Communication
3. Brain-Based Expectations
4. Sensory Overload and Shutdown
5. Emotional Regulation and Co-Regulation
6. Trauma, Grief, Attachment, and Trust
7. School vs. Home Behavior
8. De-escalation and Daily Structure
9. When to Seek Professional Help
10. What Support Planning Looks Like

When a child cannot use words to express discomfort, overwhelm, confusion, fear, or need, behavior becomes the message. This page helps caregivers practice decoding that message.

PRACTICAL CAREGIVER SKILLS

- ▶ Pause and ask: "What is my child communicating through this behavior?"
- ▶ Look for patterns rather than reacting to individual incidents in isolation.
- ▶ Consider what happened in the 30–60 minutes before the behavior began.
- ▶ Separate the behavior from the child — behavior is information, not identity.
- ▶ Practice describing the behavior neutrally before assigning meaning or blame.

⚠ Boundary
Must not imply behavior always has a clear "reason." Avoid framing that makes caregivers feel responsible for "figuring it out." This is a reframing tool, not a diagnostic guide.

⚠ If immediate danger or mental health crisis, call 988 (Suicide & Crisis Lifeline) or 911.

SOURCE NOTES (INTERNAL)

① CDC — FASD Resources: cdc.gov/fasd/resources — Family-facing FASD communication tools. Accessed 2026-05-22.

② AAP — Fetal Alcohol Spectrum Disorders: aap.org/fasd — FASD diagnosis provides a framework for understanding behavior. Accessed 2026-05-22.

③ CanFASD — Information for Families: canfasd.ca — Caregiver resources and survey data. Accessed 2026-05-22.

FASD and neurodevelopmental differences affect executive function, memory, impulse control, and emotional regulation. Help caregivers shift expectations to match developmental capacity — not chronological age.

PRACTICAL CAREGIVER SKILLS

- ▶ Match your expectations to your child's developmental level, not their age or grade.
- ▶ Assume the child is doing the best they can with the brain resources they have right now.
- ▶ Reduce verbal overload — use fewer words, visual supports, and concrete steps.
- ▶ Teach skills explicitly rather than expecting the child to "learn from consequences."
- ▶ Celebrate lagging skills as growth opportunities, not failures.

⚠ Boundary

Must not imply FASD or neurodevelopmental differences are deterministic. Avoid language suggesting children "cannot learn." Brain-based expectations are about adjusting support, not lowering hope.

SOURCE NOTES (INTERNAL)

- ① AAP — Dev/Behav Effects of FASD (1-pager): AAP PDF — FASD affects neurocognition, self-regulation, adaptive functioning. Accessed 2026-05-22.
- ② AAP — Behavioral Interventions in FASD: aap.org/fasd/interventions — Evidence-based interventions adapted for FASD strengths/weaknesses. Accessed 2026-05-22.
- ③ FASD United — Family Navigator: fasdunited.org — Helping families set realistic expectations. Accessed 2026-05-22.

INTERNAL DRAFT

Noise, transitions, crowds, clothing, touch, light, fatigue, hunger, or pain can trigger sensory overload. In these moments, behavior is nervous-system driven, not chosen.

PRACTICAL CAREGIVER SKILLS

- ▶ Notice sensory triggers by tracking what happened before difficult moments.
- ▶ Build calmer routines around known triggers — reduce errands during overwhelmed times.
- ▶ Offer sensory breaks: quiet space, weighted items, movement, headphones, fidget tools.
- ▶ During a meltdown, focus on safety and co-regulation, not teaching or discipline.
- ▶ Help the child identify their own sensory needs over time.

⚠ Boundary

Must not suggest sensory strategies replace professional evaluation. Avoid implying sensory issues affect all children the same way.

SOURCE NOTES (INTERNAL)

① AAP — Behavioral Interventions (self-regulation & neurocognition): Recommends routine, structure, and parent reframing for FASD-related sensory challenges. Accessed 2026-05-22.

② PACT (Parents and Children Together) — Emory University / AAP-referenced — "How is your engine running?" arousal state identification. Supports caregiver co-regulation and sensory awareness. Accessed 2026-05-22 via AAP.

Some children need adult nervous-system support before they can think, talk, or problem-solve. Co-regulation means staying calm together first; reasoning comes later.

PRACTICAL CAREGIVER SKILLS

- ▶ Regulate first, reason later. A dysregulated child cannot process instructions or logic.
- ▶ Model calm breathing and quiet presence before expecting the child to calm down.
- ▶ Use simple, warm language: "I'm here. You are safe. We can figure this out."
- ▶ Create a "calm-down space" that is a choice, not a punishment or time-out.
- ▶ Practice co-regulation routines during calm moments so they are familiar in hard moments.

 **Boundary**
Must not imply co-regulation always works on the first try. Avoid framing that makes caregivers feel they are "failing" if the child does not calm.

SOURCE NOTES (INTERNAL)

① AAP — Behavioral Interventions (parent reframing & skill building): Caregiver education and behavior reframing are key components of effective FASD intervention. Accessed 2026-05-22.

② CanFASD — Lifespan Intervention (Mental Health & Regulation): canfasd.ca/topics/intervention — Regulation is a core FASD support domain. Accessed 2026-05-22.

INTERNAL DRAFT

Children in foster, kinship, adoptive, or disrupted care settings may have experienced trauma, loss, or inconsistent care. Behavior may reflect survival responses, fear, or mistrust — not defiance.

PRACTICAL CAREGIVER SKILLS

- ▶ Lead with safety, predictability, and relationship before expecting behavior change.
- ▶ Build trust through consistent routines, gentle words, and follow-through.
- ▶ Recognize that grief and loss (of birth family, stability, identity) can show up as anger, withdrawal, or testing.
- ▶ Do not take behavior personally — it may be about past experience, not the current relationship.
- ▶ Some families find it helpful to seek trauma-informed therapy support when behavior reflects survival responses.

⚠ Boundary

Must not generalize trauma responses across all children. Must not imply attachment issues always require professional treatment. No DSS/court framing. No clinical directive — use "some families find it helpful" framing.

SOURCE NOTES (INTERNAL)

① FASD United — Family Navigator: fasdunited.org — Supports families navigating FASD including trauma, foster care, and adoption contexts. Accessed 2026-05-22.

② CanFASD / RCY BC — "Hands, not Hurdles": [CanFASD PDF](#) — Misunderstanding FASD leads to blaming; trauma-informed approaches are essential. Accessed 2026-05-22.

INTERNAL DRAFT

A child may hold it together at school and fall apart at home, or behave differently across settings. This is not manipulation — it reflects differing demands, supports, and emotional safety.

PRACTICAL CAREGIVER SKILLS

- ▶ Compare behavior patterns across environments without assuming the child is "choosing" where to melt down.
- ▶ Understand that home is often the safest place to release built-up stress from a demanding school day.
- ▶ Share what works at home with school; ask what works at school to try at home.
- ▶ Watch for signs of school-related stress: morning resistance, physical complaints before school, exhaustion after school.
- ▶ Advocate for school supports if behavior suggests unmet educational needs.

⚠ Boundary

Must not suggest school is always the source of all behavior. Avoid language that pits home against school.

SOURCE NOTES (INTERNAL)

① CDC — FASD Resources (parent-teacher tools): cdc.gov/fasd/resources — Communication tools between parents and teachers. Accessed 2026-05-22.

② Families Moving Forward (FMF) — AAP-referenced — Evidence-based parent coaching across home and school settings for children with FASD. Accessed 2026-05-22 via AAP.

INTERNAL DRAFT

Fewer words, calm tone, predictable transitions, visual cues, breaks, and consistent routines. Good structure reduces cognitive load and prevents many difficult moments before they start.

PRACTICAL CAREGIVER SKILLS

- ▶ Make the environment do some of the work — visual schedules, clear expectations, organized spaces.
- ▶ Use fewer words during transitions: short, concrete, calm directives.
- ▶ Build in breaks and transition warnings (visual or verbal countdown).
- ▶ During escalation: lower your voice, slow your movements, reduce demands.
- ▶ After a difficult moment, reconnect before reviewing what happened.

⚠ Boundary

Must not imply structure "fixes" behavior. Avoid framing de-escalation as a caregiver skill that always succeeds.

SOURCE NOTES (INTERNAL)

① AAP — Behavioral Interventions (structured environments): Recommends structured environments with reasonable rules, routines, and supervision for FASD. Accessed 2026-05-22.

② GoFAR Intervention — Emory University / AAP-referenced — Affective and cognitive control training for FASD. Supports principle that structure reduces cognitive load. Accessed 2026-05-22 via AAP.

INTERNAL DRAFT

Even with strong family support, some situations need professional evaluation or intervention. Recognizing these moments is a strength, not a failure.

⚠ **CRISIS — If immediate danger or mental health crisis, call 988 (Suicide & Crisis Lifeline) or 911.**

PRACTICAL CAREGIVER SKILLS

- ▶ Seek help for: safety concerns, self-harm talk, aggression that endangers family, major functional change, school exclusion.
- ▶ Seek evaluation for: suspected developmental delay, trauma history, possible FASD, or other neurodevelopmental needs.
- ▶ Reach out when caregiver feels overwhelmed, isolated, or unsure how to proceed.
- ▶ Consider support for the caregiver too — family well-being is part of the care picture.
- ▶ Know that seeking help is a sign of strength and good care, not failure.

⚠ **Boundary**

Must not provide clinical thresholds for when to seek help. Must not imply all behavior problems need professional intervention. Clear crisis language is included above. This page is educational guidance, not clinical assessment.

SOURCE NOTES (INTERNAL)

- ① CDC — "Learn the Signs. Act Early.": cdc.gov/actearly — Developmental milestone tracking and guidance on when to seek evaluation. Accessed 2026-05-22.
- ② AAP — FASD 1-Page Resource: Evidence-based treatment options exist; early diagnosis reduces risk of additional disabilities. Accessed 2026-05-22.

Evaluation is not the end point. Support may include school accommodations, therapy, caregiver education, daily routines, care coordination, and long-term planning. Families need a support map, not just a diagnosis.

PRACTICAL CAREGIVER SKILLS

- ▶ Build a support map: identify medical, educational, therapeutic, and community resources.
- ▶ Ask questions at every appointment: "What does this mean for daily life? What do we do next?"
- ▶ Connect with other families through parent support groups and peer networks.
- ▶ Plan for transitions: school changes, puberty, adulthood — each transition may need new supports.
- ▶ Take it one step at a time. Support planning is a marathon, not a sprint.

⚠ Boundary

Must not imply families must have a comprehensive plan to be "doing it right." Avoid over-promising what support systems can deliver. No referral to specific services.

SOURCE NOTES (INTERNAL)

① CanFASD — Integrated Service Delivery (Flannigan et al., 2024): canfasd.ca/topics/intervention — Person-centred coordination of health, social, and community services for individuals with FASD. Accessed 2026-05-22.

② CanFASD — C.A.R.E. Study: canfasd.ca/care-study — Caregiver perspectives on raising children with FASD; support planning is an ongoing process. Accessed 2026-05-22.

Source Reference Summary

INTERNAL NOTES

Sources used across all pages. Accessed 2026-05-22. No copied text or images.

- CDC — FASD Resources for Families**
Family-facing FASD information including communication tools for parents and teachers, problem-solving strategies. Used on Pages 2, 7.
cdc.gov/fasd/resources
- CDC — "Learn the Signs. Act Early."**
Developmental milestone tracking and guidance on when to seek evaluation. Used on Page 9.
cdc.gov/actearly
- AAP — Fetal Alcohol Spectrum Disorders (Clinical Page)**
FASD diagnosis provides framework for understanding behavior; therapeutic interventions improve outcomes. Used on Pages 2, 3.
aap.org/fasd
- AAP — Developmental & Behavioral Effects of FASD (1-Pager)**
FASD affects neurocognition, self-regulation, adaptive functioning; evidence-based treatment exists. Used on Pages 3, 9.
[AAP PDF](#)

AAP

Appendix — Tool A1: Caregiver Observation Log

DRAFT APPENDIX TOOL

Internal DraftLow RiskPending Richard Review

Purpose: Help caregivers notice patterns in behavior, setting, stressors, and what helped or did not help. Bridges Page 2 (Behavior as Communication).

⚠ Internal Use Only

Educational tool only. Not diagnostic. Not treatment plan. Not crisis plan. Not medication advice. Not referral pathway. Not a substitute for professional evaluation. **PCP / medical home remains the clinical gatekeeper.**

11 Observation Fields

01 Date · **02** Time · **03** Setting · **04** What happened before? · **05** What did your child do? · **06** Stressors or triggers · **07** What helped? · **08** What didn't help? · **09** What did you notice? · **10** Is there a pattern? · **11** Questions for care team

Summary:

Tool A1 helps caregivers notice patterns in behavior, setting, stressors, and what helped or did not help. It is an educational reflection tool only. Do not include names, DOB, insurance IDs, addresses, school names, provider names, or other identifying information.

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